

Procedure Date:
Procedure Physician:
Attending Physician/Copies To:

Carcinoma, infiltrating duct, NOS 8500/3

Site: Breast NOS C50.9

1/24/11

lw

UUID:56EFCAS8-2328-4C5C-9618-981447E76812

TCGA-BH-A1EW-01A-PR

Redacted

PATIENT HISTORY:

* nnDATE of LMP: *

DATE OF LAST DELIVERY: *

PRE-OP DIAGNOSIS: LT BR MASS

POST-OP DIAGNOSIS: SAME

OPERATIVE PROCEDURE: LT SEGM MAST-NEEDLE LOC

CLINICAL HISTORY: *

MATERIAL SUBMITTED: A) LEFT BREAST BIOPSY/MASS/SEGMENTAL MASTECTOMY, PROCUREMENT BY SURGICAL PROCEDURE
B) LEFT NEW MARGIN TISSUE, PROCUREMENT BY SURGICAL PROCEDURE
SUPERIOR

INTRAOPERATIVE CONSULTATION:

CONSULT: Left breast tissue: 11.0 by 5.0 by 4.5 cm breast tissue with 3.0 by 2.2 by 3.2 cm tumor. Superior margin grossly positive. Remaining margins free. Another firm area immediately inferior to the tumor, 0.7 by 0.5 by 0.3 cm.

Left breast, new superior margin: Segment of breast tissue, 1.0 by 8.5 by 1.3 cm. Surgical clips marks true new superior margin. Grossly negative for tumor.

ADDENDA:

Addendum

MACROSCOPIC DESCRIPTION: Block "A1" for ER/PR and Her-2/Neu.

FINAL DIAGNOSIS:

~~Signature~~

My signature is attestation that I have personally reviewed the submitted material(s) and the final diagnosis reflects that evaluation.

ER/PR

IMMUNOPEROXIDASE IDENTIFICATION OF ESTROGEN AND PROGESTERONE RECEPTORS IS CARRIED OUT ON SLIDE "A1". DISTINCT INTRANUCLEAR STAINING IS IDENTIFIED FOR ESTROGEN RECEPTOR (90%), BUT NOT FOR PROGESTERONE RECEPTOR. THEREFORE, ESTROGEN RECEPTOR IS INTERPRETED AS POSITIVE AND PROGESTERONE RECEPTOR IS INTERPRETED AS

NEGATIVE.

HER-2/NEU

c-erbB2 (HER-2/NEU) IMMUNOSTAINING IS CARRIED OUT ON MAGEE SURGICAL ~~Block "A1"~~ BLOCK "A1" (BREAST CANCER) USING A 1:300 DILUTION OF DAKO'S POLYCLONAL ANTIBODY A485 (DIRECTED AGAINST THE INTRACELLULAR DOMAIN OF c-erbB2) WITHOUT ANTIGEN RETRIEVAL. DISTINCT COMPLETE MEMBRANE STAINING IS IDENTIFIED ONLY RARELY IN 5-10% OF TUMOR CELLS. THEREFORE, c-erbB2 (HER-2/NEU) IS INTERPRETED AS NEGATIVE (SCORE 1+).

FINAL DIAGNOSIS:

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A) LEFT BREAST, SEGMENTAL MASTECTOMY:

- INFILTRATING MODERATELY DIFFERENTIATED DUCTAL AND FOCAL (APPROXIMATELY 10%) INTRADUCTAL CARCINOMA, 3.2 CM AND 0.7 CM

- NOTTINGHAM SCORE OF 7 (TUBULE FORMATION 2/3, NUCLEAR ATYPIC 3/3, MITOTIC INDEX 2/3)

- FOCAL TUMOR ASSOCIATED MICROCALCIFICATIONS

- INFILTRATING CARCINOMA IS PRESENT WITHIN <1 MM OF THE INKED SUPERIOR AND DEEP MARGINS

B) LEFT BREAST, NEW SUPERIOR MARGIN:

- BENIGN BREAST TISSUE

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		
Qualifying / Disqualifying		
Case is (circle):	YES	NO
Reviewer Initials	MS	LD